

INTRODUCTION

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Course content

- **Definition and concepts**
- **Historical Development**
- **Rationale for H/P**
- **Basic principles of H/P**
- **Approaches to H/E**

HEALTH?

Health definition...

“A state of complete physical, mental, and social well-being not merely the absence of disease or infirmity.”

World Health Organization (1948)

Physical health

- Absence of diseases or disability on the body parts .
- Anatomical integrity and the physiological well functioning of the human body
- Ability to perform routine tasks without any physical restriction.

Mental health

➤ Termed as psychological health and it is subjective sense of well being.

➤ It has two major components:

A) Cognitive component

➤ Is the ability of an individual to learn, perceive, and think clearly.

B) Emotional component

➤ Is the ability of expressing emotions (e.g. fear, happiness, and to be angry) in an “appropriate” way.

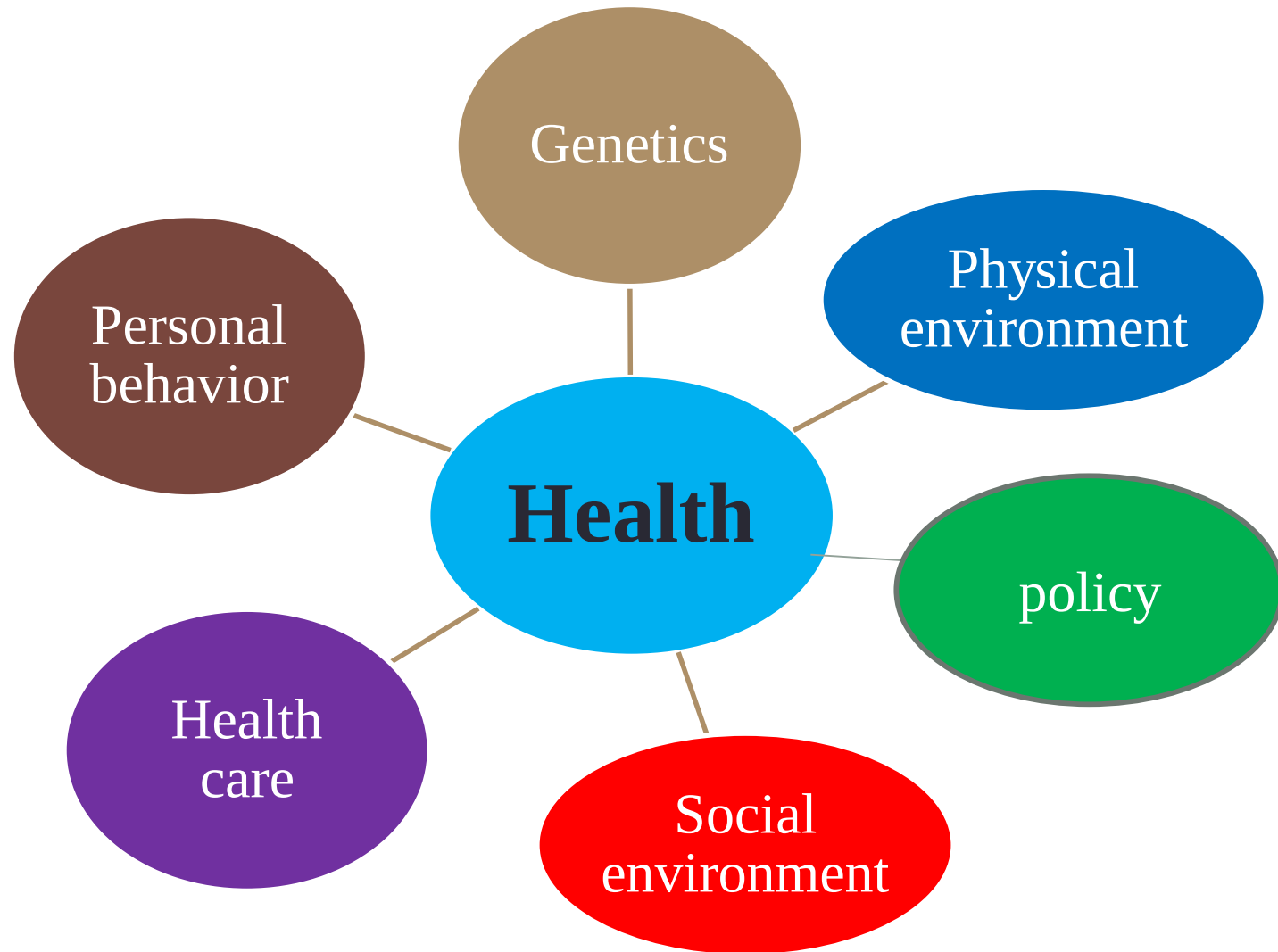
Social health

- Is the ability to **make** and **maintain** “acceptable” and “proper” interaction and communication with other people and the social environment; satisfying interpersonal relationship and role fulfillment.

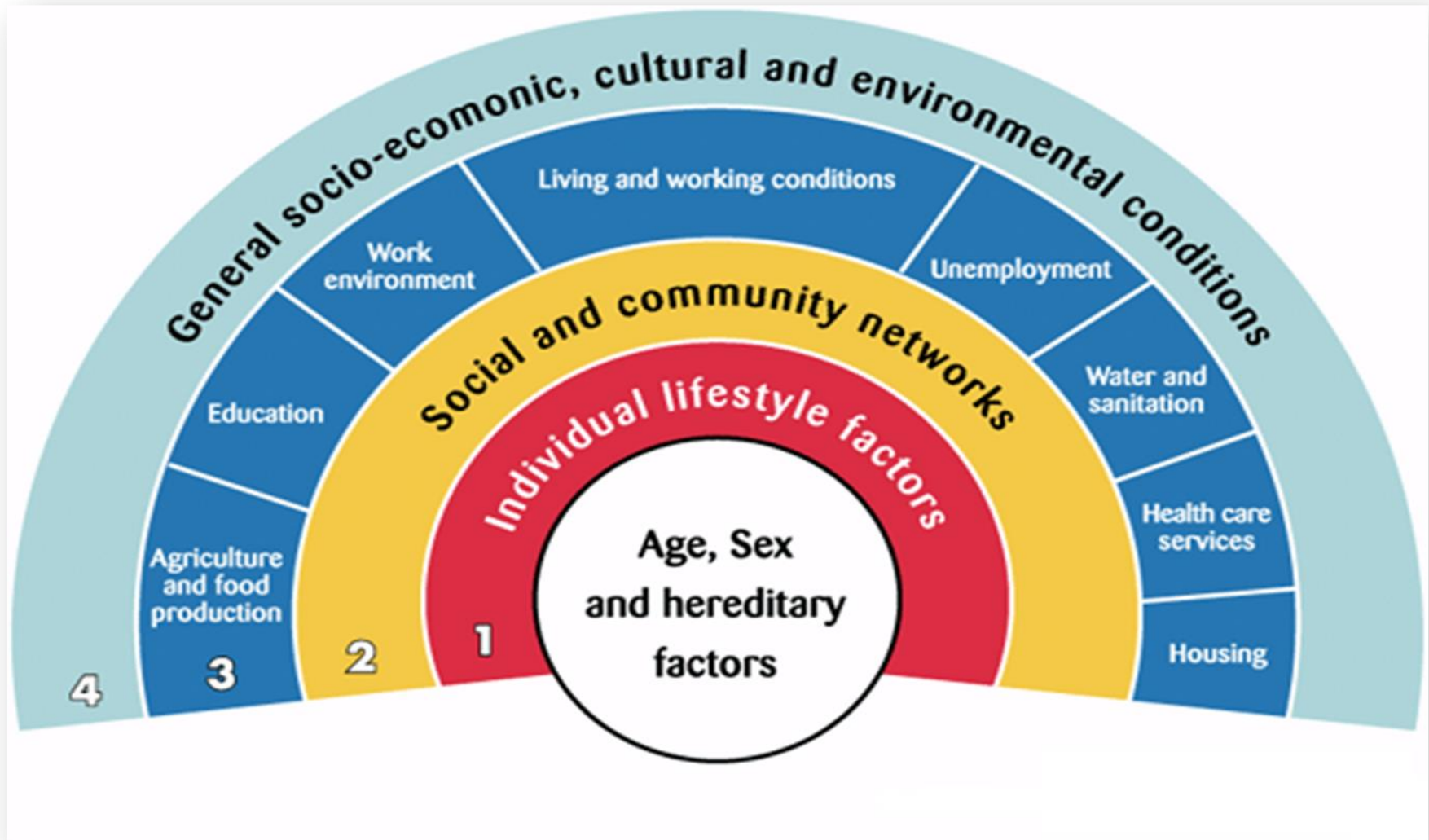
Health...

- All the above aspects of health are interrelated and interdependent. Therefore, health should be viewed as a **holistic concept**.
- The approach helps health professionals and community members to realize and act accordingly based on the multiple factors that affect health directly or indirectly.
- Limitation of WHO definition for “Health”?

Determinants of health



Multi-factorial health determinants



Determinants...Example

- Explain how multiple factors affect the health status of individuals, community and economic wellbeing of the country?
- Smoking Vs Lung cancer
- Unsafe sex Vs HIV/AIDS
- Genetics VS Cervical cancer

Definition of Health education

- “**Any combination of learning experiences designed to facilitate voluntary action conducive to health**” Lawrence Green
- **Combination** :matching multiple determinants of behaviors with multiple learning experiences or educational intervention
- **Designed** :HE is not incidental learning experiences. It is systematically planned and organized activity.
- **Facilitate**: creating favorable condition for action such as predispose, enable, reinforce.
- **Voluntary** :full understanding and acceptance of the purpose of the action.
- **Action** :measures taken by individuals, groups or community to achieve the desired health effect.

Definition ...

- H/E "comprises of consciously constructed opportunities for learning involving some form of communication designed to **improve health literacy**, including **improving knowledge**, and **developing life skills** which are conducive to individual and community health" (WHO)

Health promotion

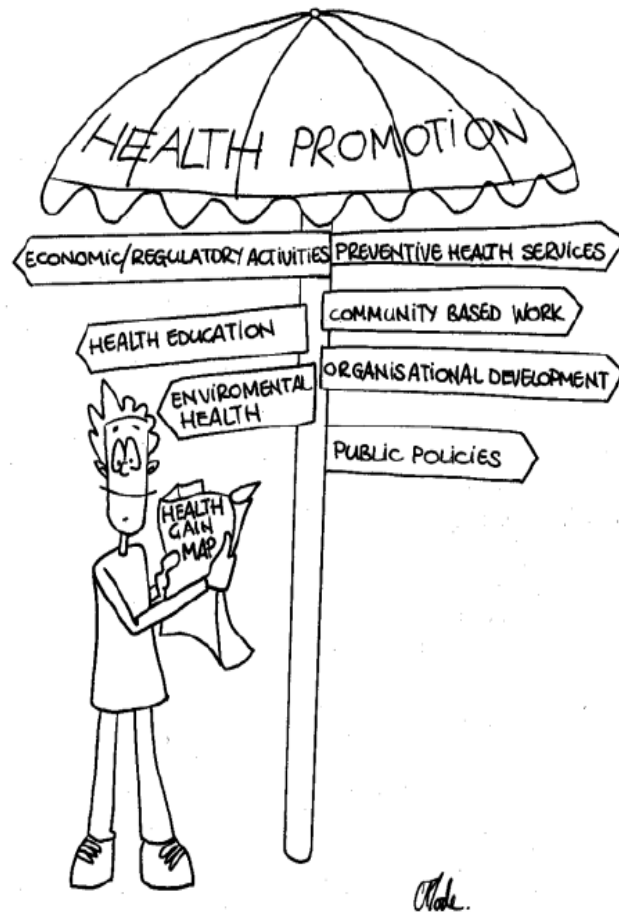
- Until 1980, HP simply referred to H/E.
- HP is a term of more recent origin than HE.

Health promotion

“any **combination** of health education and related **organizational, economic, and environmental supports** for behavior of individuals, groups, or communities **conducive to health**”

(Green and Kreuter)

Health promotion an umbrella term for a range of activities



Health promotion is an umbrella term for a range of activities

(Ewles & Simnett, 1999: 25)

Definition...

- **Combination:** Matching multiple determinants of health with multiple intervention or **sources of supports.**
- **Educational:** refers to the communication part of HP; that is HE.
- **Environmental:** refers to the social, political, and economic, organizational, policy and regulatory circumstances influence behavior or more directly health
- Create environments that support good health practices, behaviors
 - $HP = HE + \text{Environmental support}$

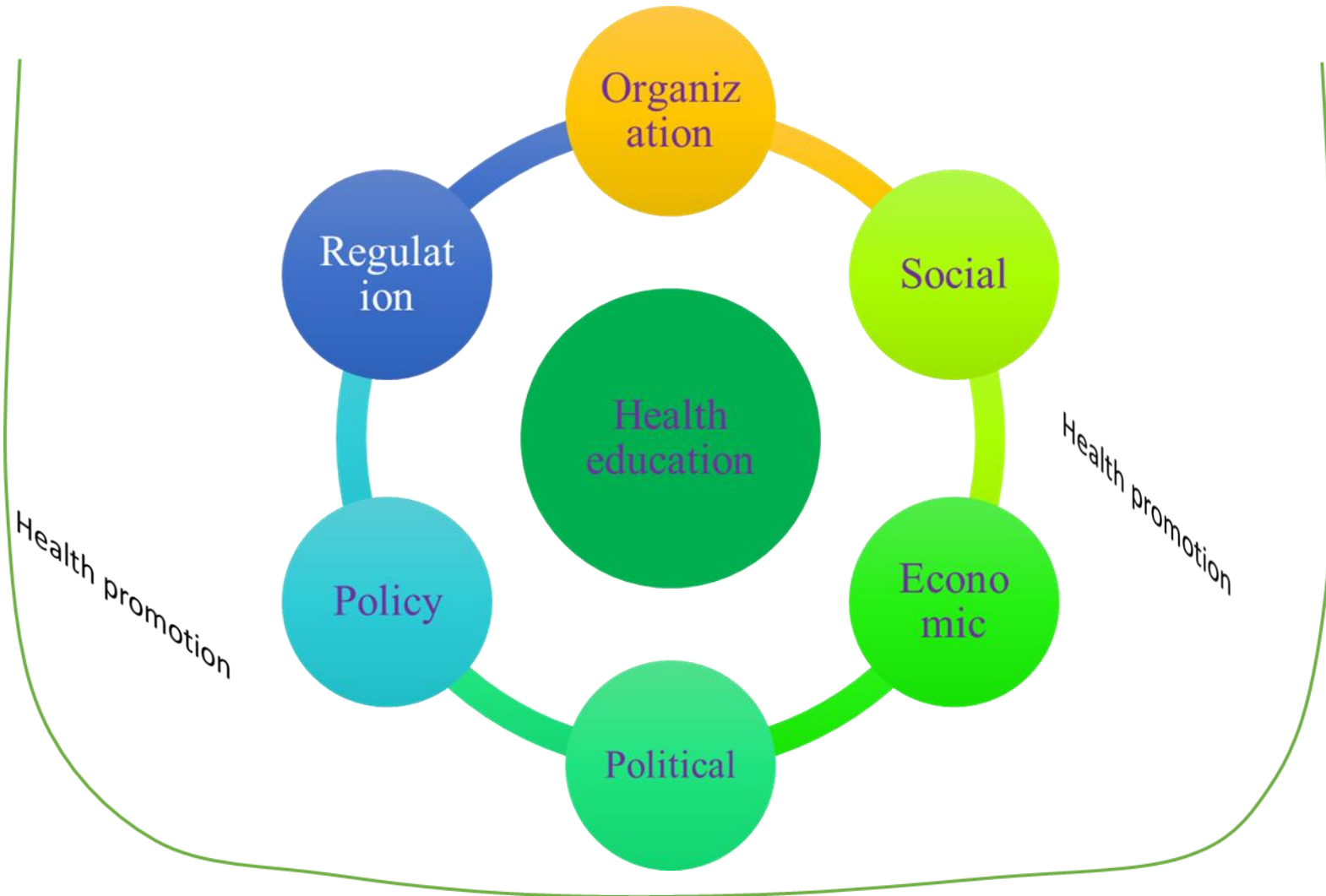
....HP(as a process)

- Health promotion is defined as “the **process** of enabling people to increase control over, and to improve, their health” (**Ottawa Charter for HP**).
- action areas identified (for people’s wellness)
 - building healthy public policy
 - creating supportive environments
 - strengthening community action
 - developing personal skills
 - re-orienting health services

The aim of health promotion

- The aim of health promotion is to reduce the underlying causes of ill-health so that there is a long-term reduction in many diseases, improve QoL at the end

The relationship between HE and HP



Health Information (HI) Vs. H/E

- HI is a series of messages transmitted to the public by knowledgeable professionals.
- H/I is blind to the reasons behind behavior
- Appropriate for newly emerging health problems and during epidemics

Historical Development of HE /P

- As a practice as old as human being
- As a profession in the world ~>100 years – USA
- Health belief model 1966: the oldest model
- As a course: Gondar Health science ~ 1954
- As a profession in Ethiopia: JU 2006

Principles of health promotion

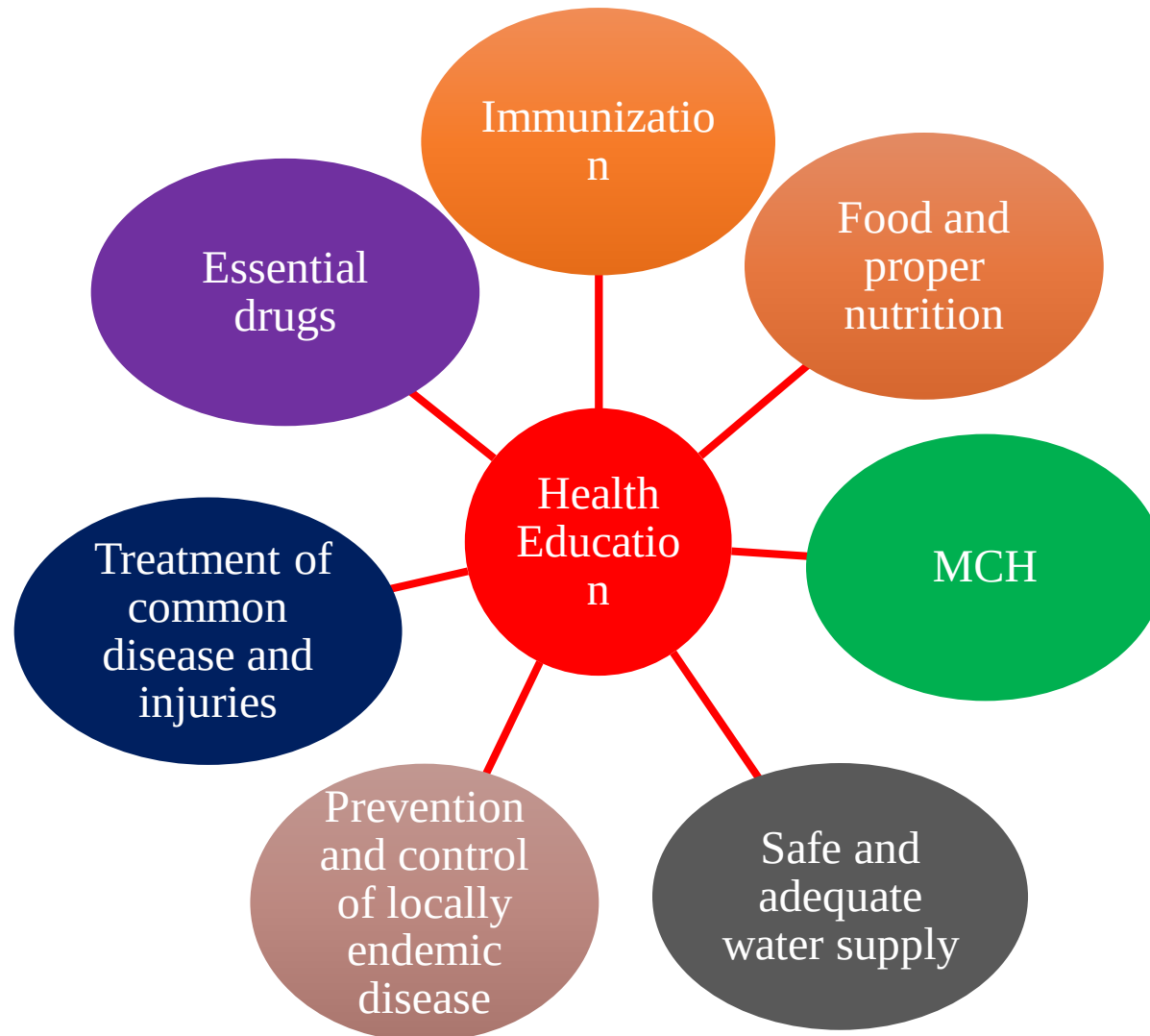
1. Address the wider determinants of health
2. Base activities on the best available evidence
3. Act to reduce inequalities in health
4. Ensure active community participation
5. Empower individuals
6. Explicitly consider difference in gender and culture
7. Facilitate intersectoral co-operation

What health promotion is not ?

- Public relation
- Advertising
- Patient education
- Propaganda

Role of HE/P in HC

Maximize provision, utilization, accessibility and acceptability



Major health problems

Developing countries

- Under weight
- unsafe sex
- Unsafe H₂O, sanitation & hygiene
- Zinc, Fe, vit A deficiency
- Indoor smoke from solid fuels
- High blood pressure
- Tobacco

Developed countries

- High blood pressure
- Tobacco
- Alcoholism
- High cholesterol
- High BMI
- Low fruit & vegetable intake
- Physical inactivity
- Illicit drugs etc

Assumptions of health education

1. Health status can be changed
2. Disease occurrence theories and principles can be understood
3. Appropriate prevention strategies can be developed to deal with the identified health problems
4. An individual's health is affected by a variety of factors
5. Change in individual and societal health behaviors and lifestyles will affect an individual's health status positively

Con't...

6. Health behavior changes occur in stages
7. Individuals, families, small groups, and communities can be taught to assume responsibility for their health, which in turn changes their health behaviors and lifestyles
8. Individual responsibility should not be viewed as victim blaming
9. Health behavior change is a process, not an event

Dimensions and characteristics of health education

1. Concerned with the whole person and encompasses physical, mental, social, aspects.
2. Touches upon many fields
3. A lifelong process from birth to death, helping people to change and adapt at all stages
4. Concerned with people at all points of health and illness, from the completely healthy to the clinically sick and handicapped.

Con't...

5. Concerned with helping people to help themselves: **voice of their own problem**
6. Involves formal and informal teaching and learning using a range of methods
7. Concerned with a range of goals
8. The general value of health as a community asset has increased and governments are "willing" to spend more on health care

Rationale for health E/P

1. The continued existence and spread of CDs
2. For some diseases H/E is the only practical choice?
3. The tendency of increasing magnitude of chronic conditions
4. Human behaviors are almost the single causes for the development of most health conditions .
5. Wide spread of unhealthy behaviors
6. people's ignorance of causes of their illnesses
7. Increased awareness of people on chronic health problems and the need to know preventive actions (right to health and right to information)

Principles of health education

1. All HE should be need-based.
2. HE should not become an artificial situation or formal teaching-learning.
3. It is necessary to discuss freely on the health problems and the solution and to ensure that all the good and bad points, advantages and disadvantages, difficulties, etc., are thoroughly dealt with.

Con't...

6. There should be a free flow of communication.
7. The health educator has to make himself acceptable.
8. The health educator should not only have correct information with him on all matters that he has to discuss but also should himself practice what he professes.

Con't...

9. The health educator has to adjust his talk and action to suit the group for whom he has to give HE.
10. A health educator has to employ all possible methods of education.
11. the health educator should as far as possible make every effort to reason out and rationalize, so that the client is able to internalize the relevant ideas.

Con't...

- 12. A well planned program is necessary.
- 13. The use of audio-Visual aids for support and reinforcement is of particular significance in HE
- 14. HE should be specific and relevant to the problems and the available solution.
- 15. find out the existing behavioral patterns and their basis.

Con't...

16. There is always some perception or belief, etc... which should be studied first before HE can be started.

17. The process of education should be done step by step and with due attention to the different principles of communication.

Con't...

18. HE should be able to provide an opportunity for the client or clients to go through the stages of identification of problems, planning, implementation evaluation and so on.

19. The health educator should use terms which can be understood.

20. HE should start from the existing culture and gradually try at change of habit and practices. Moreover, the HE effort should aim at small changes in a graded fashion and not be too ambitious.

Principle....summary

- Principle of educational Dx
- Principle of Participation
- Principle of multiple methods
- Principle of planning and organizing
- Facts: H/E should be based on scientific findings/facts and current knowledge
- Cultural acceptance
- Need based

Approaches to health education

1. Persuasive approach

- attempt to influence the other persons to do what we want them to do

2. Empowerment approach

- giving people information, problem solving and decision making skills to make decisions but leaving the actual choice to the people.

Health education in the level of disease prevention

1. Primary health education

➤ Is comprised of those preventive measures that forestall the onset of illness or injury during the pre-pathogenesis period (before the disease process begins),

e.g.

- Wearing safety belt
- Immunization
- Physical exercise
- Breast feeding

H/E in level of diseases prevention...

2. Secondary health education

➤ aimed at promoting early diagnosis and prompt treatment of a disease to cure or to limit disability and prevent more serious pathogenesis

e.g.

- breast-cancer screening
- Blood pressure examination
- Cholesterol level examination
- treating malaria patients.

H/E in level of diseases prevention...

3. Tertiary health education

- work to retain, reeducate, and rehabilitate the individual who has already incurred disability, impairment, or dependency,
- Social and psychological adjustment

e.g.

- educating on ART adherence PLWHA
- Working with the diabetes individual to ensure the daily Injections are taking

Thank you!

“The deviation of man from the state in which he was originally placed seems to have proved to him a prolific source of diseases.”

Edward Jenner

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